Day 01: 01/12/2025 (Monday)

The intern reported to the psychiatric department and attended the morning rounds with the doctors. During rounds, she observed the evaluation of inpatients and listened carefully to discussions regarding diagnosis, symptom severity, and medication adjustments. She walked through the wards and closed units to understand patient safety procedures and ward management. In the casualty department, she observed detailed case history taking and learned how to record chief complaints, duration of illness, and treatment history. She interacted with nurses to gather information about patients' sleep patterns, appetite, medication compliance, and behavior. She also spoke with family members and bystanders to obtain background details. In the afternoon, she engaged patients in shuttle and chess to build rapport and encourage social interaction.

Day 02: 02/12/2025 (Tuesday)

The intern joined morning rounds and observed follow-up assessments. She noted how doctors assessed improvement and modified treatment plans. In casualty, she conducted supervised case history interviews, focusing on personal history and psychosocial stressors. She walked through wards and interacted with patients to observe mood and communication. She consulted nurses regarding behavioral changes. She discussed cases with co-interns for better understanding. In the afternoon, she organized badminton and table tennis activities. A doctor conducted a teaching session explaining mood disorders and related medical terminology.

Day 03: 03/12/2025 (Wednesday)

The intern attended rounds and observed cases of psychosis and depression. She carefully documented symptoms discussed by the doctor. In casualty, she conducted structured interviews and performed mental status examinations under supervision. She interacted with caregivers to obtain family psychiatric history. A social worker conducted a class explaining psychosocial assessment and structured case history format. In the afternoon, she engaged patients in chess and shuttle to improve concentration and cooperation.

Day 04: 04/12/2025 (Thursday)

The intern observed medication review during rounds and learned about dosage adjustments. She walked through the wards and interacted individually with patients to build therapeutic rapport. She collected case history details regarding past illness and treatment compliance. She consulted nurses for daily functioning updates. In the afternoon, she conducted table tennis and group games. A doctor provided a class on anxiety disorders and diagnostic criteria.

Day 05: 05/12/2025 (Friday)

The intern observed detailed case presentations during morning rounds. In casualty, she focused on substance use history and withdrawal symptoms while taking case history. She interacted with family members about relapse triggers. She discussed cases with co-interns. In the afternoon, she organized group recreational activities. A weekly seminar on mental health awareness was conducted, and she participated in presentation and discussion.

Day 06: 06/12/2025 (Saturday)

The intern attended morning rounds and observed emergency psychiatric admissions in the casualty. She carefully noted how acute agitation and aggressive behavior were managed. She observed the doctor explaining risk assessment and immediate stabilization procedures. After rounds, she walked through both open and closed wards and observed patient monitoring systems, safety precautions, and staff coordination. She interacted with nurses to understand how they record patient behavior, sleep cycle, appetite changes, and response to medication. In casualty, she conducted detailed case history taking under supervision, focusing on onset, precipitating factors, and past psychiatric history. She interacted with family members to gather information about previous hospitalizations and treatment compliance. She discussed her findings with co-interns and clarified doubts with the supervising doctor. In the afternoon, she engaged patients in chess and shuttle games to reduce restlessness and improve attention span. She also encouraged quiet interaction sessions for withdrawn patients.

Day 07: 08/12/2025 (Monday)

During morning rounds, the intern observed follow-up reviews of patients admitted for mood disorders. She carefully noted the discussion regarding medication adjustments and side effects. She walked through the wards and spoke with patients individually to assess their emotional state and level of insight. She interacted with nurses to obtain behavioral updates during night shifts. In casualty, she conducted structured case history interviews, including personal history, educational history, occupational functioning, and family relationships. She discussed complex cases with co-interns for academic sharing. A social worker conducted a class explaining family dynamics and the importance of psychosocial background in diagnosis.

In the afternoon, she organized badminton and table tennis sessions to improve physical activity and social bonding among patients.

Day 08: 09/12/2025 (Tuesday)

The intern attended rounds and observed newly admitted psychotic patients. She learned about symptom identification such as delusions, hallucinations, and disorganized behavior. She documented mental status findings. In casualty, she explored detailed symptom history and duration of illness. She interacted with caregivers to understand the patient's functioning before illness onset. She consulted nurses regarding medication adherence and daily hygiene patterns. A doctor conducted a class explaining psychotic disorders and antipsychotic medications. The intern took notes on medical terminology and diagnostic criteria. In the afternoon, she encouraged patients to participate in group games and observed improvement in cooperation and communication.

Day 09: 11/12/2025 (Thursday)

The intern observed medication response evaluation during rounds. She noted improvement and residual symptoms in different patients. She walked through closed wards and observed safety supervision measures. In casualty, she conducted mental status examinations focusing on mood, thought content, perception, and cognition. A social worker class was conducted on ethical communication and confidentiality in psychiatry. She interacted with family members to understand caregiving burden and emotional stress. Afternoon sessions included shuttle and relaxation exercises for anxious patients.

Day 10: 12/12/2025 (Friday)

The intern observed anxiety and panic disorder cases during rounds. She learned how doctors differentiate between generalized anxiety and panic attacks. In casualty, she explored stressors, coping mechanisms, and personality traits. She interacted with nurses regarding sleep disturbances and appetite issues. A doctor conducted a session explaining substance use disorders and withdrawal symptoms. The intern discussed case interpretations with co-interns. Afternoon recreational activities included badminton and chess to improve focus and reduce anxiety

Day 11: 13/12/2025 (Saturday)

The intern attended morning rounds and observed detailed discussions on bipolar disorder cases. The doctor explained the difference between manic and depressive episodes and how mood stabilizers are adjusted based on symptom severity. She carefully observed how doctors assessed speech pattern, energy level, and insight in patients. After rounds, she walked through the wards and interacted with patients to understand their current mood and cooperation level. She spoke with nurses to collect updates about sleep disturbances, appetite, and medication compliance. In the casualty department, she conducted structured case history interviews focusing on onset of illness, triggering factors, and family psychiatric history. She interacted with caregivers to understand relapse patterns and support systems at home. Later, she discussed the cases with co-interns and clarified doubts with the supervising doctor. In the afternoon, she organized shuttle and table tennis activities to improve social interaction. A weekly seminar on mental health awareness was conducted, and she actively participated in presentation and discussion.

Day 12: 14/12/2025 (Sunday)

The intern joined morning rounds and observed emergency admissions with acute agitation and severe anxiety symptoms. She noted how doctors assessed risk factors and decided on immediate treatment plans. She walked through the closed ward to observe safety precautions and staff supervision. She interacted with nurses regarding high-risk patient monitoring and behavioral observation charts. In casualty, she conducted detailed case history taking, focusing on personal history, occupational functioning, and substance use patterns. She spoke with family members to understand psychosocial stressors and previous treatment history. She discussed these findings with co-interns to improve diagnostic understanding. In the afternoon, she conducted chess and relaxation activities for patients to reduce restlessness and improve attention.

Day 13: 15/12/2025 (Monday)

During morning rounds, the intern observed schizophrenia cases and noted both positive and negative symptoms. The doctor explained the importance of long-term medication adherence. She interacted with patients individually to assess their thought process and level of insight. She spoke with nurses to understand daily functioning and hygiene maintenance of patients. In casualty, she conducted mental status examinations under supervision and documented findings carefully. A social worker conducted a class on psychosocial rehabilitation and community reintegration. The intern took notes and clarified doubts. In the afternoon, she organized badminton and group interaction activities to enhance cooperation and social skills.

Day 14: 16/12/2025 (Tuesday)

The intern attended rounds and observed personality disorder cases. The doctor explained behavioral patterns and interpersonal difficulties. She carefully observed patient interaction during ward visits. She interacted with nurses to gather information about mood fluctuations and behavioral changes. In casualty, she explored developmental history and family relationships while taking case history. She discussed diagnostic impressions with co-interns. Later, a doctor conducted a teaching session on personality disorders and diagnostic criteria. In the afternoon, she engaged patients in shuttle and table tennis games to encourage teamwork and enjoyment.

Day 15: 17/12/2025 (Wednesday)

The intern participated in rounds and observed anxiety spectrum cases. She learned how to differentiate generalized anxiety from panic disorder. She walked through wards and spoke with patients to assess worry patterns and physical symptoms. She consulted nurses about sleep patterns and appetite changes. In casualty, she conducted supervised mental status examinations and documented cognitive findings. A social worker class focused on communication skills and structured documentation in psychiatry. In the afternoon, she organized chess and relaxation exercises to help patients improve concentration.

Day 16: 18/12/2025 (Thursday)

The intern attended rounds and observed obsessive-compulsive disorder cases. The doctor explained compulsions, obsessions, and behavioral therapy principles. She interacted

with patients to understand the impact of symptoms on daily functioning. She consulted nurses regarding medication side effects. In casualty, she collected detailed case histories including past medical history and treatment compliance. She discussed findings with co-interns for academic understanding. In the afternoon, she conducted badminton and shuttle games to improve physical activity and reduce anxiety.

Day 17: 19/12/2025 (Friday)

The intern observed follow-up reviews during rounds and noted improvement in symptom management. She interacted with family members regarding medication adherence and relapse prevention. In casualty, she focused on psychosocial stressors and coping mechanisms during interviews. Weekly seminar preparation was done, and she assisted in presenting a topic related to mental health awareness. She discussed clinical insights with co-interns. Afternoon group games were organized to promote cooperation and improve mood among patients.

Day 18: 20/12/2025 (Saturday)

The intern attended rounds and observed crisis intervention procedures for aggressive patients. She noted how de-escalation techniques were applied. She walked through closed wards and observed safety supervision. She interacted with nurses about observation charts and patient monitoring. In casualty, she documented detailed case summaries. She discussed challenging cases with co-interns. In the afternoon, she conducted chess and quiet group interaction sessions to reduce agitation and promote calm behavior.

Day 19: 22/12/2025 (Monday)

The intern observed depressive relapse cases during rounds. She learned about suicide risk assessment and warning signs. She carefully observed how doctors explored suicidal ideation. She interacted with nurses regarding emotional changes and appetite disturbances. In casualty, she conducted structured interviews focusing on mood symptoms and psychosocial background. A social worker class discussed ethical responsibilities and confidentiality in psychiatry. Afternoon activities included shuttle and group games to encourage positive engagement.

Day 20: 23/12/2025 (Tuesday)

The intern attended rounds and observed neurocognitive disorder cases. The doctor explained memory assessment and cognitive decline. She interacted with caregivers to understand daily functioning and caregiving burden. She consulted nurses regarding medication compliance. In casualty, she conducted cognitive screening interviews under supervision. A teaching session by the doctor explained medical terminology related to neurocognitive disorders. In the afternoon, she organized badminton and table tennis sessions to improve coordination and social interaction.

Day 21: 24/12/2025 (Wednesday)

The intern observed chronic schizophrenia management during rounds. She noted the importance of rehabilitation planning. She walked through wards and assessed patient social interaction levels. She interacted with nurses for behavioral updates. In casualty, she conducted

detailed occupational history assessment. She discussed diagnostic impressions with co-interns. Afternoon games such as chess and shuttle improved concentration and cooperation.

Day 22: 26/12/2025 (Friday)

The intern attended rounds and observed discharge planning discussions. She learned about family counseling and follow-up care. She interacted with family members to understand home support systems. Weekly seminar on mental health awareness was conducted, and she actively presented and participated in discussion. She documented case summaries carefully. Afternoon recreational sessions encouraged teamwork and enjoyment among patients.

Day 23: 28/12/2025 (Sunday)

The intern observed emergency admissions and acute psychiatric symptoms. She carefully documented presenting complaints and precipitating factors. She interacted with caregivers for immediate history collection. She consulted nurses regarding stabilization procedures. In casualty, she conducted supervised interviews focusing on stress and coping. Afternoon sessions included relaxation exercises and table tennis to reduce stress.

Day 24: 29/12/2025 (Monday)

The intern attended rounds and observed long-term inpatient evaluations. She learned about rehabilitation and occupational therapy planning. A social worker class discussed

community resources and patient reintegration. She interacted with nurses regarding progress notes and behavioral observation. In casualty, she completed detailed psychosocial documentation. In the afternoon, she organized shuttle and group games to promote interaction and improve mood.

Day 25: 30/12/2025 (Tuesday)

On the final day, the intern attended morning rounds and reviewed the progress of several patients she had previously interviewed. She discussed case summaries with doctors and clarified doubts regarding diagnosis and treatment planning. She interacted with nurses to understand overall ward functioning and patient improvement. She spoke with family members to understand their experience and support system. She completed pending case histories and reviewed documentation with co-interns. In the afternoon, she conducted final recreational activities including chess and badminton. She reflected on the knowledge gained in case history taking, patient interaction, ward observation, diagnostic understanding, and therapeutic communication during the entire internship period.

Case History 1

Socio-Demographic Details

Name: AK

Sex: Male

Age: 29

Education: 12th Standard

Occupation: Mechanic

Marital Status: Unmarried

Socioeconomic Status: Lower

Religion: Hindu

Residential Area: Semi-urban

Family Type: Nuclear

Information

Informant: Elder brother

Reliability: The information provided by the informant appeared reliable and consistent.

Adequacy: The information obtained was adequate for assessment.

Chief Complaints with their duration

- History of excessive talkativeness (3 Weeks)
- Decreased need for sleep, irritability, and impulsive behaviour (3 Weeks).

- Expressed ideas such as believing that he is God and that he possesses superpowers (2 Weeks).
- Reports a belief that others can identify or hear his thoughts (2 Weeks).
- Aggressive and harmful behaviour towards family members, along with increased substance use (1 Week).

History of Present Illness

The client was reportedly functioning adequately until three weeks prior to admission. Following emotional distress related to a romantic relationship failure, he began exhibiting behavioural changes. Initially, he showed increased energy, reduced need for sleep (sleeping only 1–2 hours per night), and excessive talking. His speech became rapid, unclear, and difficult to interrupt. Gradually, he developed grandiose delusions, repeatedly stating that he is God and that he has supernatural powers. He expressed firm belief that others could identify or hear his thoughts, suggestive of thought broadcasting. His behaviour became increasingly impulsive, including verbal aggression, throwing objects, and threatening family members. Substance use increased significantly during this period. Due to escalating aggression, harmful behaviour, and severe sleep disturbance, he was admitted for inpatient psychiatric care one week ago. There is no past history of psychiatric illness or treatment.

Predisposing Factors

The predisposing factors include a family history of anxiety symptoms in the mother and a history of alcohol dependence in the father, who expired eight years ago, suggesting possible genetic and environmental vulnerability. Substance use since high school further increased the risk for psychological difficulties. Additionally, a lower socioeconomic background may have contributed to chronic stress and limited access to supportive resources.

Precipitating Factors

The precipitating factors include a love failure following a romantic relationship breakup, which appears to have triggered significant emotional stress. This emotional distress likely contributed to the onset or exacerbation of the current episode.

Perpetuating Factors

The perpetuating factors include ongoing drug dependence, which continues to aggravate symptoms. Poor coping strategies and lack of insight into the illness further maintain the condition. Limited psychosocial support also reduces opportunities for recovery and reinforces the persistence of difficulties.

Mode of Onset of Illness: Acute

Course of Illness: Episodic, currently in manic phase

Biological Functions or Associated Disturbances

Regarding biological functions and associated disturbances, there is a markedly decreased need for sleep and an increased activity level. Increased substance use has been observed, along with poor self-care during the acute episode, indicating significant disruption in daily functioning.

Negative History

- No history suggestive of seizure disorder
- No history of head injury
- No chronic medical illness
- No previous psychiatric illness

History of Past Illness

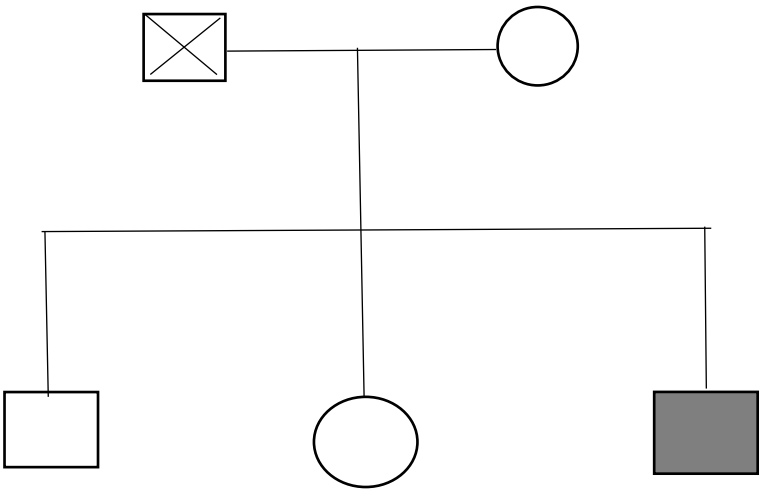
Past Psychiatric History: No past psychiatric history reported.

Medical History: No significant medical illness reported.

Family History

The client belongs to a nuclear family. His father had alcohol dependence and passed away 8 years ago. His mother has a history of anxiety symptoms but has not undergone formal psychiatric treatment. The client has a brother and a sister. Both are married. Brother has one male child and Sister Has both male and female childrens. He was the youngest child in the family.

Genogram



Personal History

Birth History

No significant complications during birth reported. Normal Delivery and weight during birth approximately 3 kg.

Milestone History

Developmental milestones were within normal limits.

Childhood History

No major behavioural disturbances reported.

Normal and Calm atmosphere during childhood.

Substance use began during high school years.

Educational History

Completed 12th standard.

Academic performance was poor.

Not interested for Study and Fail 12th std due to increased usage of drugs.

Occupational History

Age of starting work: Early twenties

Jobs held: Occasional daily wage labour

Work Satisfaction, Competence: Irregular employment due to substance use and behavioural issues

Type of relationship with superiors and colleagues: Frequent conflicts

Future ambitions: Unrealistic grand plans during current episode

Marital History: Unmarried.

Sexual History

Age of Attaining Knowledge: Adolescence

Mode of Gaining Knowledge: Peers

No reported sexual dysfunction

Increased sexual drive during current episode

Alcohol and Substance History

History of drug use since high school (mainly cannabis and alcohol). Gradual progression to dependence. Increased use during stress and significantly escalated during current episode. No previous sustained abstinence reported.

Premorbid Personality

Attitude to others: Social but impulsive

Attitude to self: Average self-confidence

Moral and religious attitudes: Mildly religious

Predominant Mood: Generally cheerful but emotionally reactive

Leisure activities: Socializing with peers

Reaction pattern to stress: Substance use and irritability

Mental Status Examination

General Appearance and Behavior

Rapport: Initially difficult to establish

General Appearance: Poor grooming

Grooming: Inadequate

Dressing: Bright and inappropriate

Gait and Posture: Restless and overactive

Attitude towards examiner: Overfamiliar and occasionally irritable

Psychomotor Activity: Increased

Speech

Rate and quality of speech: Increased rate; pressured and unclear at times

Volume and tone of speech: Loud and forceful

Flow and rhythm of speech: Increased flow with flight of ideas

Mood and Affect

Subjective: Reports feeling powerful and superior

Objective: Euphoric and irritable

Affect appears labile and expansive

Thought

Stream of Thought

Flight of ideas: Present

Thought Blocking: Absent

Perseveration: Absent

Neologisms: Absent

Form of Thought

Speed of thoughts: Increased

Loose associations: Absent

Circumstantial thoughts: Absent

Tangential thoughts: Present occasionally

Content of Thought

Grandiose delusions (claims of being God and possessing superpowers)

Delusion of thought broadcasting (belief that others can identify his thoughts)

No suicidal ideation reported

Perception: No clear hallucinations elicited during examination.

Orientation

Place: Oriented

People: Oriented

Time: Oriented

Attention and Concentration: Distractible

Memory

Immediate: Intact

Recent: Intact

Remote: Intact

Intelligence

General Knowledge: Adequate

Comprehension: Adequate

Arithmetic: Adequate

Abstraction: Adequate

Judgment

Personal Judgment: Impaired

Social Judgment: Impaired

Test Judgment: Impaired

Insight

Grade-3 Level: Awareness of being sick but attributes problems to external stressors and denies need for treatment

Diagnostic Formulation

The client is a 29-year-old unmarried male from a lower socioeconomic background presenting with acute onset of elevated mood, decreased need for sleep, increased psychomotor activity, pressured and excessive speech, grandiose delusions, thought broadcasting, impulsive and harmful behaviour, and increased substance use. There is no past psychiatric history. Family history is significant for anxiety in mother and alcohol dependence in father. The

overall clinical picture is suggestive of Bipolar Affective Disorder, current episode manic with psychotic symptoms.

Provisional Diagnosis with ICD Code

Bipolar Affective Disorder, Current Episode Manic with Psychotic Symptoms – F31.2

Differential Diagnosis

Substance-Induced Psychotic Disorder

Schizoaffective Disorder

Acute and Transient Psychotic Disorder

Case History 2

Socio-Demographic Details

Name: AKK

Sex: Male

Age: 24 years

Education: Higher Secondary completed

Occupation: Employed (previously working in Dubai)

Marital Status: Unmarried

Socioeconomic Status: High

Religion: Hindu

Residential Area: Kochi

Family Type: Nuclear (living with parents and brother)

Information

Informant: Mother

Reliability: The information provided by the informant appeared clear and reliable.

Adequacy: Information obtained was adequate for assessment.

Chief Complaints with Duration

- Aggressive behaviour, intense anger outbursts (2 Weeks)
- Decreased need for sleep, increased energy levels, excessive activity, and suspiciousness towards his romantic partner (past few months).

- He has been destructive and uncontrollable aggression (3 Weeks).
- Increased eating habits, heightened sexual drive, and inability to sit idle (1 Week)

History of Present Illness

The client was reportedly functioning well until a few months ago. He completed his higher secondary education with good academic performance and later went to Dubai for employment. Work satisfaction was reported to be good. He returned to Kochi after being contacted by his former classmate to begin a romantic relationship and live together. After a few months into the relationship, he developed suspicious thoughts that his partner was involved with other men. He began restricting her movements and refused to allow her to attend work. During this period, he developed drug dependence and exhibited intense anger outbursts. He threw her mobile phone, damaged their car, and behaved aggressively. A legal complaint was filed against him. Following this, his aggression escalated at home, where he began damaging household items. He showed markedly decreased need for sleep, remained energetic throughout the day, and was unable to sit idle. He displayed increased appetite and heightened sexual interest. His speech and behaviour were suggestive of increased psychomotor activity. Due to worsening aggression, sleeplessness, and behavioural disturbances, he was admitted for psychiatric evaluation.

Predisposing Factors

The predisposing factors include high emotional involvement in a romantic relationship, which may have increased vulnerability to emotional instability. Substance use also contributed as a risk factor. In addition, possible personality vulnerabilities such as impulsivity and jealousy traits may have predisposed the individual to difficulties in managing emotions and relationships.

Precipitating Factors

The precipitating factors include ongoing relationship conflicts and suspicion toward the partner, which appear to have triggered the current episode. The legal case filed against him may have further intensified stress and emotional reactivity, contributing to the onset of symptoms.

Perpetuating Factors

The perpetuating factors include ongoing drug use, which continues to worsen behavioural and emotional disturbances. Poor anger management and relationship instability further maintain the difficulties. Poor insight into behavioural changes also prevents recognition of the problem and delays appropriate intervention.

Mode of Onset of Illness: Subacute

Course of Illness: Continuous worsening over months

Biological Functions or Associated Disturbances

Regarding biological functions and associated disturbances, there is a markedly decreased need for sleep, increased appetite, heightened sexual drive, elevated energy levels, and noticeable restlessness, all of which indicate significant changes in physiological and behavioural functioning.

Negative History

- No history of seizures
- No head injury
- No chronic medical illness
- No past psychiatric admission

History of Past Illness

Past Psychiatric History: No previous psychiatric illness reported.

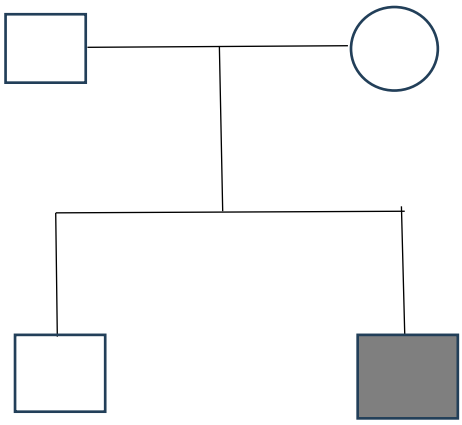
Medical History

Born by cesarean section; no complications reported. No significant medical illnesses.

Family History

Lives with father, mother, and brother. No history of major psychiatric illness reported in family members.

Genogram



Birth History

Born by cesarean section. No perinatal complications reported. Birth Weight Was 3 kg.

Milestone History

Developmental milestones achieved within normal limits.

Childhood History

Reported good behaviour during childhood. No behavioural disturbances.

Educational History

Performed well academically during school and higher secondary education.

Well attendive and bright in studies.

Occupational History

Age of starting work: Early twenties

Jobs held: Employed in Dubai

Work Satisfaction: Reported good job satisfaction

Relationship with colleagues: Stable

Future ambitions: Career-oriented prior to onset

Marital History

Unmarried. History of live-in relationship with classmate, which became conflictual.

Sexual History

Age of attaining knowledge: Adolescence

Mode of gaining knowledge: Peers

Increased sexual drive during current episode

No reported sexual dysfunction

Alcohol and Substance History

History of drug use during recent months. Substance use escalated during relationship conflicts. No long-term documented history before this episode.

Premorbid Personality

Attitude to others: Friendly and well-adjusted

Attitude to self: Confident

Moral and religious attitudes: Average

Predominant mood: Cheerful

Leisure activities: Social interactions

Reaction to stress: Anger and impulsivity

Mental Status Examination

General Appearance and Behaviour

Rapport: Initially cooperative but irritable

General Appearance: Well-built and appropriately dressed

Grooming: Adequate

Dressing: Appropriate

Gait and Posture: Restless

Attitude towards examiner: Guarded at times

Psychomotor Activity: Increased

Speech

Rate and Quality: Increased rate

Volume and Tone: Loud

Flow and Rhythm: Rapid flow; goal-directed; no marked flight of ideas but increased productivity

Mood and Affect

Subjective: Reports feeling energetic and justified in his actions

Objective: Irritable and expansive

Affect: Labile

Thought

Stream of Thought

Flight of ideas: Not prominent

Thought blocking: Absent

Perseveration: Absent

Form of Thought

Speed of thoughts: Increased

Loose associations: Absent

Tangentiality: Occasionally present

Content of Thought

Suspiciousness regarding partner's fidelity

Ideas of jealousy

No clear grandiose delusions reported

No suicidal ideation

Perception: No hallucinations reported.

Orientation: Oriented to time, place, and person.

Attention and Concentration: Distractible

Memory

Immediate: Intact

Recent: Intact

Remote: Intact

Intelligence

General knowledge: Adequate

Comprehension: Adequate

Arithmetic: Adequate

Abstraction: Adequate

Judgment

Personal Judgment: Impaired

Social Judgment: Impaired

Test Judgment: Impaired

Insight

Grade 2 Level: Partial insight; acknowledges anger issues but does not recognize severity of illness.

Diagnostic Formulation

The client is a 24-year-old unmarried male from a high socioeconomic background presenting with decreased need for sleep, increased energy, heightened sexual drive, aggressive behaviour, impulsivity, suspiciousness towards partner, increased psychomotor activity, and substance use. The clinical presentation is suggestive of a manic episode, possibly substance-induced, requiring further evaluation.

Provisional Diagnosis (ICD-10)

Bipolar Affective Disorder, Current Episode Manic – F31.1

(To rule out Substance-Induced Mood Disorder)

Differential Diagnosis

Substance-Induced Mood Disorder

Delusional Disorder (Jealous Type)

Intermittent Explosive Disorder

Case History 3

Socio-Demographic Details

Name: S D

Sex: Male

Age: 42 years

Education: MBA Graduate

Occupation: Corporate employee

Marital Status: Married

Socioeconomic Status: High

Religion: Christian

Residential Area: Kakkanad

Family Type: Nuclear

Information

Informant: Wife

Reliability: The information provided by the informant appeared reliable and consistent.

Adequacy: The information obtained was adequate for assessment.

Chief Complaints with Duration

- Long-standing history of alcohol use, inability to abstain from alcohol, shivering and irritability when not consuming alcohol (Since Childhood)

- Increased alcohol intake following his mother's death, and recent medical complications including liver damage (3 years)
- Repeated episodes of drinking directly from the office without returning home and progressive deterioration in family and occupational functioning (3 months)

History of Present Illness

The client has a chronic history of alcohol consumption beginning in late adolescence. Early exposure to alcohol occurred within the family environment, as both parents consumed alcohol regularly. During college days, alcohol consumption increased, and he also began smoking. After marriage, occupational stress increased significantly. Alcohol use became a daily habit, and he began consuming alcohol directly from his workplace before returning home. Over time, he developed tolerance and required increasing quantities of alcohol. Following the death of his mother, alcohol intake intensified markedly as a coping mechanism for stress and loneliness. He reported inability to function without alcohol. When abstinent, he experienced withdrawal symptoms such as tremors (shivering), irritability, restlessness, and discomfort, which compelled him to resume drinking. Medical evaluation revealed liver damage associated with prolonged alcohol use. Due to progressive dependence, physical complications, and inability to control drinking behaviour, he was admitted for inpatient management.

Predisposing Factors

The predisposing factors include early exposure to alcohol within the family environment, which may have normalized substance use from a young age. Parental alcohol use further contributed to this vulnerability. The individual also reports perceived emotional neglect during childhood, which may have affected emotional development and coping abilities. Limited maternal affection due to the mother working away from home and an

emotionally distant father may have contributed to unmet emotional needs and difficulties in forming secure attachments later in life.

Precipitating Factors

The precipitating factors include increased occupational stress after marriage, which may have heightened emotional strain and vulnerability. The death of the mother appears to have been a significant triggering event, leading to intense grief. Feelings of loneliness and emotional distress further contributed to the onset or worsening of the condition.

Perpetuating Factors

The perpetuating factors include physical dependence on alcohol, which maintains continued use. Withdrawal symptoms such as tremors and irritability reinforce repeated consumption to relieve discomfort. A habitual daily drinking pattern, ongoing work-related stress, and poor stress coping mechanisms further sustain the problem over time.

***Mode of Onset of Illness*:** Gradual

Course of Illness: Chronic and progressively worsening

Biological Functions or Associated Disturbances

the individual experiences alcohol withdrawal symptoms, including tremors and irritability. Liver damage has been reported as a medical complication. Sleep is disturbed in the absence of alcohol, and increased tolerance has developed, indicating the need for higher amounts of alcohol to achieve the same effects

Negative History

- No history of psychotic symptoms
- No history of seizure disorder (except risk during withdrawal not reported)

- No other substance dependence except nicotine
- No past psychiatric admission reported

History of Past Illness

Past Psychiatric History: No documented past psychiatric illness.

Medical History

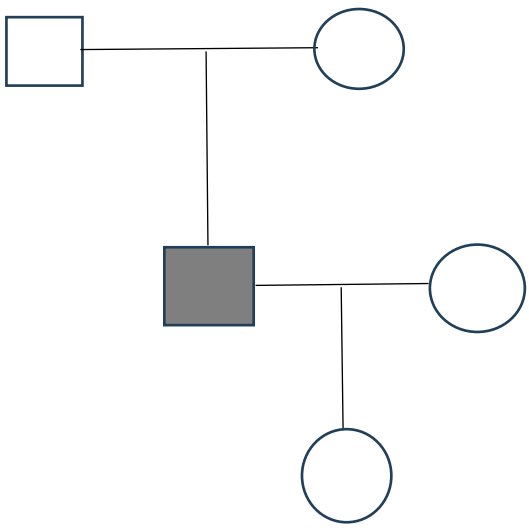
C-section birth; no perinatal complications.

Chronic alcohol use leading to liver damage.

Family History

Born in a nuclear family. Both father and mother consumed alcohol regularly. Mother dead. He was the only child in the family. He was married and they had a daugher. Childhood marked by reduced parental supervision and emotional warmth. Other family members were involved in caregiving.

Genogram



Personal History

Birth History

Born by cesarean section. No birth complications reported.

Milestone History

Developmental milestones within normal limits.

Childhood History

Reported lack of adequate maternal affection due to mother working away from home. Father emotionally less involved. Care provided mainly by other relatives.

Educational History

Completed MBA. Academically competent.

Occupational History

Age of starting work: Early twenties

Jobs held: Corporate employment

Work Satisfaction: Initially good; later stress increased

Relationship with colleagues: Professional

Future ambitions: Career stability and financial growth

Marital History: Married. Marital strain due to chronic alcohol use and behavioural changes.

Sexual History

No significant sexual dysfunction reported.

Alcohol and Substance History

Alcohol use began in adolescence with gradual progression to daily use. Developed tolerance and physical dependence. Withdrawal symptoms present when alcohol is stopped. Increased use after mother's death. Smoking habit initiated during college and continues. No other substance use reported.

Premorbid Personality

Attitude to others: Social and responsible

Attitude to self: Ambitious and achievement-oriented

Moral and religious attitudes: Average religious background

Predominant mood: Generally stable

Leisure activities: Social drinking

Reaction to stress: Uses alcohol as coping mechanism

Mental Status Examination

General Appearance and Behaviour

Rapport: Cooperative

General Appearance: Well-built and appropriately dressed

Grooming: Adequate

Dressing: Formal and neat

Gait and Posture: Normal

Attitude towards examiner: Cooperative but mildly anxious

Psychomotor Activity: Mildly increased during withdrawal phase

Speech

Rate and Quality: Normal

Volume and Tone: Normal

Flow and Rhythm: Coherent and goal-directed

Mood and Affect

Subjective: Reports feeling stressed and unable to manage without alcohol

Objective: Irritable and anxious

Affect: Congruent with mood

Thought

Stream of Thought

Normal

Form of Thought

Logical and goal-directed

Content of Thought

Preoccupation with alcohol use and stress

No delusions or suicidal ideation reported

Perception: No hallucinations reported

Orientation: Oriented to time, place, and person

Attention and Concentration: Intact

Memory

Immediate: Intact

Recent: Intact

Remote: Intact

Intelligence

General knowledge: Above average

Comprehension: Adequate

Arithmetic: Adequate

Abstraction: Adequate

Judgment

Personal Judgment: Impaired regarding alcohol use

Social Judgment: Partially impaired

Test Judgment: Adequate

Insight

Grade 5 Level: Partial insight- acknowledges alcohol problem but feels unable to quit due to dependence

Diagnostic Formulation

The client is a 42-year-old married male with early exposure to alcohol, chronic progressive alcohol use, physical dependence with withdrawal symptoms, and alcohol-related liver damage. The course is chronic and worsening, with significant psychosocial and medical complications.

Provisional Diagnosis (ICD-10)

Alcohol Dependence Syndrome – F10.2

Alcohol-Induced Liver Disease (medical comorbidity)

Differential Diagnosis

Harmful Use of Alcohol

Alcohol-Induced Mood Disorder

Generalized Anxiety Disorder secondary to substance use

Case History 4

Socio-Demographic Details

Name: BL

Sex: Female

Age: 22 years

Education: Completed 12th Standard

Occupation: Homemaker

Marital Status: Married

Socioeconomic Status: Below average

Religion: Hindu

Residential Area: Kochi

Family Type: Nuclear

Information

Informant: Husband

Reliability: Information provided appears reliable.

Adequacy: Information obtained was adequate for assessment.

Chief Complaints with Duration

- Unstable interpersonal relationships, frequent conflicts with husband, excessive use of social media leading to multiple romantic interactions, self-harming behaviour during

emotional distress, intense fear of abandonment, and attention-seeking behaviour (2–3 years).

History of Present Illness

The client was reportedly functioning adequately during adolescence. After completing her 12th standard education, she entered into a romantic relationship which later resulted in marriage. However, over the past few years, she has developed unstable interpersonal patterns characterized by multiple online romantic interactions through social media platforms. She frequently engages in chatting and emotional involvement with other males despite being married. When confronted by her husband regarding these interactions, she becomes emotionally dysregulated, leading to intense quarrels. During minor conflicts, she exhibits self-harming tendencies such as superficial cutting and threatening self-harm. These behaviours are usually triggered by fear of rejection, criticism, or perceived neglect by her husband. She demonstrates attention-seeking behaviours, expressing a desire to be noticed and admired by other men. Her emotional states fluctuate rapidly from anger and irritability to sadness and emptiness. She often reports feeling lonely and misunderstood.

Predisposing Factors

The predisposing factors include a below average socioeconomic background, which may have contributed to long-term stress and limited emotional support. There is also a possibility of unmet emotional needs during the developmental years, which could have influenced personality formation and coping styles. Early romantic involvement and insecure attachment patterns may have further increased vulnerability to emotional distress in intimate relationships.

Precipitating Factors

The precipitating factors include ongoing marital conflicts that may have triggered the current difficulties. Perceived rejection or criticism from the husband appears to have intensified emotional distress. Additionally, social media interactions that reinforce validation-seeking behaviour may have acted as immediate triggers for the present condition.

Perpetuating Factors

The perpetuating factors include excessive social media usage, which continues to maintain the problematic behaviour. Maladaptive coping strategies and emotional dysregulation further sustain the difficulties. Reinforcement of attention-seeking behaviour through external validation also contributes to the persistence of symptoms over time.

Mode of Onset of Illness: Gradual

Course of Illness: Chronic and fluctuating

Biological Functions or Associated Disturbances

The individual experiences sleep disturbances during periods of emotional distress and occasional loss of appetite during conflicts. However, no major physical illness has been reported.

Negative History

- No history of psychotic symptoms
- No history of bipolar mood episodes
- No history of substance abuse
- No neurological illness

History of Past Illness

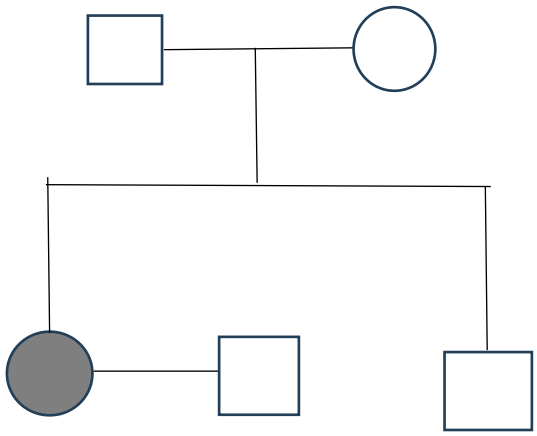
Past Psychiatric History: No previous psychiatric admissions reported.

***Medical History*:** No significant medical illness reported.

Family History

The Client comes from a nuclear family with her parents and a younger brother. Lives with husband. No reported family history of major psychiatric illness.

Genogram



Personal History

Birth History

No reported complications during birth.

Milestone History

Developmental milestones achieved appropriately.

Childhood History

Reported average upbringing. Emotional needs may not have been consistently met.

Educational History

Completed 12th standard. Academic performance average.

Occupational History

Currently a homemaker.

Marital History

Married to a man she previously loved. Relationship characterized by instability, frequent quarrels, jealousy issues, and emotional intensity.

Sexual History

Age of attaining knowledge: Adolescence

Mode of gaining knowledge: Peers and social media

Reports strong need for validation and admiration

No reported sexual dysfunction

Substance Use History

No significant substance use reported.

Premorbid Personality

Attitude to others: Emotionally dependent and sensitive to rejection

Attitude to self: Low self-esteem with need for external validation

Moral and religious attitudes: Average

Predominant mood: Emotionally unstable

Leisure activities: Excessive social media usage

Reaction pattern to stress: Self-harm threats, emotional outbursts, impulsive behaviour

Mental Status Examination

General Appearance and Behaviour

Rapport: Established with mild emotional lability

General Appearance: Appropriately dressed

Grooming: Adequate

Gait and Posture: Normal

Attitude towards examiner: Cooperative but emotionally reactive

Psychomotor Activity: Within normal limits

Speech

Rate and Quality: Normal but becomes rapid when emotionally distressed

Volume and Tone: Variable depending on emotional state

Flow and Rhythm: Coherent and relevant

Mood and Affect

Subjective: Reports feeling empty, lonely, and misunderstood

Objective: Labile affect

Affect is intense and rapidly shifting

Thought

Stream of Thought

Logical and goal-directed

Form of Thought

No loosening of associations

Content of Thought

Fear of abandonment and Preoccupation with relationships. No delusions and no suicidal intent at present but history of self-harm gestures

Perception: No hallucinations reported

Orientation: Oriented to time, place, and person

Attention and Concentration: Intact

Memory

Immediate: Intact

Recent: Intact

Remote: Intact

Intelligence

General knowledge: Adequate

Comprehension: Adequate

Abstraction: Adequate

Judgment

Personal Judgment: Impaired during emotional distress

Social Judgment: Impaired in interpersonal situations

Test Judgment: Adequate

Insight

Grade 3 Level: Partial insight; acknowledges emotional problems but minimizes severity of self-harming behaviour

Diagnostic Formulation

The client is a 22-year-old married female presenting with unstable interpersonal relationships, intense fear of abandonment, impulsive involvement in multiple online romantic relationships, recurrent self-harming behaviour during emotional distress, identity instability, emotional lability, and attention-seeking behaviour. The symptoms are chronic and pervasive across situations, significantly impairing marital functioning.

Provisional Diagnosis (ICD-10)

Emotionally Unstable Personality Disorder – Borderline Type (F60.31)

Differential Diagnosis

Histrionic Personality Disorder

Bipolar Affective Disorder (to be ruled out)

Adjustment Disorder

Case History 5

Socio-Demographic Details

Name: JJ

Sex: Male

Age: 26 years

Education: Undergraduate (Incomplete Degree)

Occupation: Unemployed

Marital Status: Unmarried

Socioeconomic Status: Middle

Religion: Christian

Residential Area: Urban area

Family Type: Nuclear

Information

Informant: Father

Reliability: Information provided appears reliable.

Adequacy: Information obtained was adequate for assessment.

Chief Complaints with Duration

- Intense drug use for several years
- Social withdrawal, reduced speech, decreased motor activity (4 months)

- Expressing firm belief that he is chosen to protect the world from disasters placed by God for the (past 1 year, with worsening over the last 6 months).

History of Present Illness

The client was reportedly functioning normally during late adolescence. He began experimenting with drugs (mainly cannabis and synthetic substances) during college, which gradually progressed to regular and intense use. Approximately one year ago, family members noticed behavioural changes. He became socially withdrawn, reduced interaction with family, and spent most of his time alone writing extensively in notebooks. He wrote multiple pages describing himself as a special person chosen to protect the world from disasters created by God. He believed he possessed a divine mission and supernatural abilities. He also wrote an elaborate autobiography describing himself and his girlfriend in unrealistic and exaggerated ways, intending to present it to her mother. The content was described as illogical and grandiose. Over the past six months, his speech became reduced in quantity, and he spoke only when prompted. Psychomotor activity decreased, and he appeared slowed and indifferent. Personal care declined. He demonstrated poor motivation and emotional expression. There is no clear history of manic episodes. Substance use continued throughout the course of illness. Due to progressive functional decline, delusional beliefs, and reduced self-care, he was brought for psychiatric evaluation.

Predisposing Factors

history of substance abuse, particularly chronic drug use, which may have contributed to the onset and progression of the condition. There is also a possible genetic vulnerability, although the family history is not significant at present and requires further exploration. In addition, psychosocial stressors related to a romantic relationship appear to have played a role in predisposing the individual to the current difficulties.

Precipitating Factors

The precipitating factors include increased drug use, which may have triggered or worsened the symptoms. Academic stress along with the discontinuation of studies also appears to have contributed significantly to the onset of the condition. Additionally, interpersonal stress played an important role in precipitating the current episode.

Perpetuating Factors

The perpetuating factors include ongoing substance abuse, which continues to aggravate the condition. Poor insight into the illness further maintains the problem, as it may reduce treatment adherence. Social isolation and a lack of a structured daily routine also contribute to the persistence and worsening of symptoms over time.

Mode of Onset of Illness: Insidious

Course of Illness: Gradual and progressively deteriorating

Biological Functions or Associated Disturbances

The individual has experienced a disturbed sleep pattern and occasional reduction in appetite. Poor self-care has also been noted. However, no major physical illness has been reported.

Negative History

- No history of head injury
- No seizure disorder
- No history suggestive of bipolar disorder
- No significant medical comorbidity reported

History of Past Illness

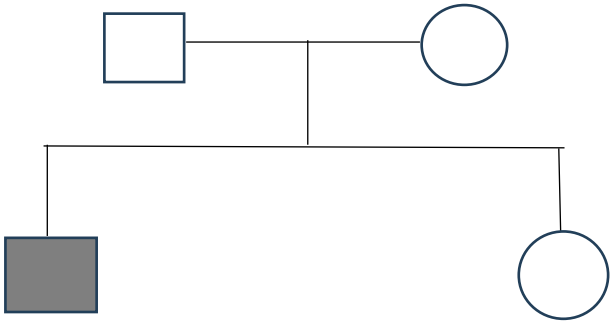
Past Psychiatric History: No prior psychiatric admission reported.

Medical History: No major medical illness reported.

Family History

The Clients father was born in a 2-child family and mother from a 2-child family. The Client was the first child and have a younger sister. There is no marked childhood abnormalities and other problems were found.

Genogram



Personal History

Birth History

Full-term normal delivery. No complications reported.

Milestone History

Developmental milestones within normal limits.

Childhood History

Average development, no behavioural issues reported.

Educational History

Average academic performance. Dropped out of undergraduate program due to drug use and behavioural changes.

Occupational History

Age of starting work: Not employed consistently

Jobs held: Occasional part-time work

Work Satisfaction: Poor due to lack of motivation

Future ambitions: Unrealistic and grandiose plans during illness

Marital History

Unmarried. Had a romantic relationship; wrote exaggerated autobiographical material regarding himself and girlfriend.

Sexual History

No significant abnormalities reported.

Substance Use History

Chronic drug use (primarily cannabis and other psychoactive substances) since college years. Usage increased over time. Likely contributing to onset and exacerbation of psychotic symptoms.

Premorbid Personality

Attitude to others: Previously social

Attitude to self: Average self-esteem

Moral and religious attitudes: Moderate religious belief

Predominant mood: Calm

Leisure activities: Socializing with peers prior to illness

Reaction pattern to stress: Drug use

Mental Status Examination

General Appearance and Behaviour

Rapport: Difficult to establish

General Appearance: Poor grooming

Gait and Posture: Slowed movements

Attitude towards examiner: Guarded

Psychomotor Activity: Decreased

Speech

Rate and Quality: Decreased rate; reduced spontaneity

Volume and Tone: Low volume

Flow and Rhythm: Delayed responses; poverty of speech

Mood and Affect

Subjective: States he feels “chosen”

Objective: Blunted affect

Affect incongruent at times

Thought

Stream of Thought

Poverty of thought present

Form of Thought

Logical at times but slowed

Content of Thought

Grandiose delusion (belief of being chosen to protect the world from disasters placed by God)

Overvalued autobiographical writings with unrealistic content

Religious delusion

No clear suicidal ideation

Perception: No clear hallucinations elicited during interview, though history suggestive of possible internal preoccupation.

Orientation: Oriented to time, place, and person

Attention and Concentration: Impaired

Memory

Immediate: Intact

Recent: Intact

Remote: Intact

Intelligence

General knowledge: Average

Comprehension: Adequate

Judgment

Personal Judgment: Impaired

Social Judgment: Impaired

Test Judgment: Impaired

Insight

Grade I – Complete denial of illness

Diagnostic Formulation

The client is a 26-year-old unmarried male with chronic substance use presenting with insidious onset of social withdrawal, reduced speech, decreased psychomotor activity, poor self-care, grandiose religious delusions, unrealistic autobiographical writings, and functional deterioration over one year. The clinical picture is suggestive of a psychotic disorder with prominent negative symptoms.

Provisional Diagnosis (ICD-10)

Schizophrenia – F20.9

(To rule out Substance-Induced Psychotic Disorder)

Differential Diagnosis

Substance-Induced Psychotic Disorder

Schizoaffective Disorder

Delusional Disorder